PROCEDURE: Esophagogastroduodenoscopy (Upper GI Endoscopy) with narrow-band imaging (NBI) and Seattle protocol four-quadrant mucosal biopsies.

REASON FOR PROCEDURE: A 62-year-old male with a 12-year history of gastroesophageal reflux disease (GERD) presenting with new-onset progressive solid-food dysphagia and persistent epigastric heartburn despite twice-daily proton pump inhibitor therapy.

PROCEDURE DETAILS: Following informed consent, the patient received topical Cetacaine oral spray and deep sedation with intravenous propofol. An Olympus GIF-H190 high-definition gastroscope was passed under direct visualization through the cricopharyngeus without resistance.

Esophagus: The proximal and mid-esophagus showed normal squamous mucosa without strictures or rings. At the distal esophagus, beginning at 34 cm from the incisors, salmon-pink colored velvety mucosa was visualized extending circumferentially to the gastroesophageal junction located at 38 cm (Prague Criteria: C4 M5). Detailed mucosal assessment under Narrow-Band Imaging (NBI) and dual focus revealed regular mucosal and vascular pit patterns without focal nodularity or mucosal ulceration. In accordance with the Seattle biopsy protocol, jumbo cold forceps biopsies were taken in four quadrants every 2 cm throughout the Barrett's segment (total 8 specimens).

Stomach: Retroflexion demonstrated a 3 cm sliding hiatal hernia. The gastric fundus, body, and incisura angularis revealed mild, diffuse patchy mucosal erythema without active bleeding or ulcerations. Multiple antral biopsies were procured for rapid urease testing (CLOtest) and Helicobacter pylori histological evaluation.

Duodenum: The duodenal bulb and second portion of the duodenum exhibited normal mucosal folds without duodenitis, celiac-type scalloping, or mass lesions.

IMPRESSION:

1. Long-segment Barrett's esophagus (Prague classification C4 M5) without visible endoscopically resectable dysplastic nodules.

2. Moderate 3 cm sliding hiatal hernia.

3. Mild erythematous antral gastritis; CLOtest pending.

4. No mechanical esophageal obstruction identified; dysphagia likely secondary to reflux-induced motility dysfunction versus microscopic inflammatory changes.